

MASTER COURSE TEXTBOOK • THEMATIC STUDY

MEDIEVAL MEDICINE IN BRITAIN (c1250–c1500)*Supernatural Orthodoxy, The Galenic Monopoly & The Black Death*

The History Department

GCSE MASTERCLASS SERIES • C1250–C1500



Archival Photographic Plate • Contemporary Primary Record • Medieval Medicine in Britain

CORE SPECIFICATION ENQUIRIES

- **L1:** Supernatural & Religious Explanations
- **L2:** Rational Ideas: Galen & The Four Humours
- **L3:** Approaches to Treatment & Prevention
- **L4:** Medical Providers & Monastic Hospitals
- **L5:** Case Study: The Black Death (1348–49)

HISTORICAL MECHANISMS & CONCEPTS

- **Ecclesiastical Monopoly:** Scriptoria & Papal authority
- **Teleology:** Purposeful divine bodily design
- **Humoural Balance:** Opposites, phlebotomy & purging
- **Hospital Care:** *Care Not Cure* monastic regimes
- **Astrological Miasma:** 1345 Planetary conjunction

EDEXCEL PAPER 1 EXAMINATION BLUEPRINT

- **Q3:** Similarity / Difference across Eras [4m]
- **Q4:** Causal Analysis / Explain Why [12m]
- **Q5/Q6:** 16-Mark Synoptic Essay [+4 SPaG]
- **Assessment Objective 1:** Precise recall of detail
- **Assessment Objective 2:** Analytical historical causation

KT1.1: Supernatural & Religious Explanations of Disease (c1250–c1500)

Specification Anchor: Ideas about the cause of disease and illness: supernatural and religious explanations; the influence of the Church on medicine and science.

The Catholic Church Monopoly

[1.1] In medieval England (c1250–c1500), the Roman Catholic Church exercised near-total control over intellectual life, education, and social values. Formal education was an ecclesiastical privilege; cathedral schools and universities like Oxford and Paris were staffed entirely by clergy. Most crucially, book production was concentrated inside monastic scriptoria, where monks hand-copied manuscripts. Because every text required months of laborious transcription, the Church acted as an unyielding censor, deciding which books were preserved and which were suppressed.

[1.2] Medical teaching was strictly theoretical and anchored to Latin scripture. The Church taught that the physical world was merely a temporary, imperfect realm governed by divine will. Consequently, understanding the universe did not require physical dissection or laboratory experiments; it required studying holy scripture and classical treatises approved by the Pope. To question established teaching was not merely an academic error—it was an act of heresy against God.

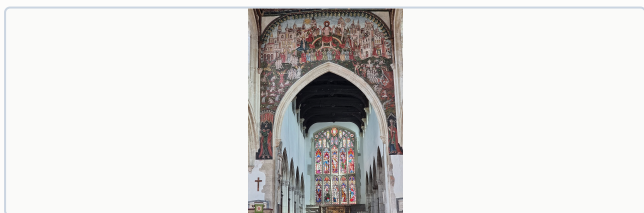
God's Wrath, Sin & Leprosy

[2.1] Because the Church taught that God controlled every aspect of health, sickness was universally interpreted as a spiritual condition. Sudden illness was seen as divine retribution—a punishment sent by God for personal sins such as greed, gluttony, or heresy, or as a divine test of a Christian's faith and devotion (similar to Job in the Old Testament). Recovery was regarded as a miracle granted through prayer, confession, and repentance, rather than the result of medical skill.

[2.2] The most vivid illustration of divine punishment was leprosy. Mentioned prominently in the Bible, leprosy was regarded as a physical manifestation of inner moral corruption and sin. Sufferers were socially ostracised; they underwent a symbolic funeral service before being banished to specialized Lazar houses built outside city walls. When forced to travel, lepers had to wear concealing grey cloaks, keep to the middle of the road, and continuously ring a handheld bell while crying out for alms, warning healthy citizens to avoid their breath.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

The Doom Painting of St Thomas's Church, Salisbury (c.1475)



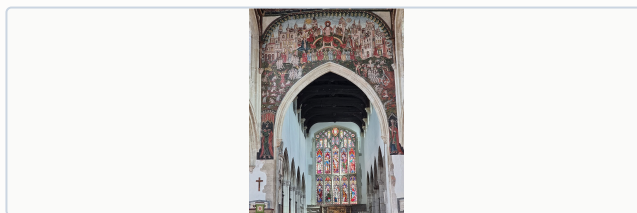
"The monumental 15th-century Doom painting above the chancel arch depicting Christ in Majesty, the righteous ascending to Heaven (left), and the damned being dragged into the Hellmouth (right)."

Provenance: St Thomas's Church, Salisbury, c.1475 wall mural above the chancel arch.

Hinge Question: Study Christ judging humanity and the damned being dragged into the Hellmouth on the right. How did medieval wall paintings like this reinforce the belief that sickness and epidemics were sent by God as punishments for sin?

SOURCE B HISTORICAL PRIMARY EVIDENCE c1250–c1500

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KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS

the medieval Catholic Church hold a monopoly on medical knowledge: The Church controlled all universities and monastic scriptoria, where monks hand-copied books and strictly censored ideas that contradicted scripture.

medieval people explain the religious cause of disease: Illness was viewed as divine retribution sent by God to punish sin, test faith, or cleanse the soul; recovery was achieved through prayer and repentance.

Lazar houses: Segregated quarantine colonies built outside town boundaries to isolate leprosy sufferers, who were forced to wear cloaks and ring warning bells.

CORE VOCABULARY

Celestial Forces & Astrology

[3.1] Alongside divine retribution, medieval people believed that human health was intimately governed by the alignment of the stars and planets. While early Church leaders initially condemned astrology as pagan fortune-telling, by the 14th century, astrological medicine was fully incorporated into university curricula and endorsed by European prelates. The cosmos was viewed as God's vast clockwork creation; planets were believed to exert physical gravitational and mystical influences upon the earthly elements.

[3.2] When catastrophic epidemics struck, physicians pointed to unusual celestial configurations. Most famously, when the Black Death decimated Europe, scholars at the University of Paris officially declared that the plague was caused by the conjunction of Saturn, Jupiter, and Mars in the astrological sign of Aquarius in March 1345. This planetary alignment was believed to have drawn up poisonous vapors from deep within the earth, corrupting the air with deadly miasma.

Galen's Teleology & The Enforcement of Dogma

[4.1] Why did the Christian Church adopt the medical treatises of Claudius Galen, a pagan doctor from 2nd-century imperial Rome? The crucial connection was teleology. Galen repeatedly argued in his treatises that every single bone, muscle, and organ in the human body had been purposefully crafted by a supreme, singular Creator. This design argument aligned so perfectly with Genesis and Christian creation theology that church leaders declared Galen's anatomical writings infallible sacred truth.

[4.2] The consequence for European medicine was catastrophic stagnation. Questioning Galen became synonymous with questioning scripture. When rare university dissections of executed criminals were conducted, the physician sat high up in a raised chair reading aloud from Galen's Latin text while an untrained barber cut the body; if a physical organ contradicted Galen, the corpse was dismissed as deformed. When the English Franciscan friar Roger Bacon argued around 1277 that scholars should stop relying on ancient authority and conduct firsthand empirical experiments, church authorities had him imprisoned. For over a millennium, medical progress was effectively criminalised.

KEY FIGURE

c.1214–1292



Roger Bacon

FRANCISCAN SCHOLAR & NATURAL PHILOSOPHER

Early advocate of empirical scientific observation who was imprisoned by the Catholic Church for questioning ancient authority.

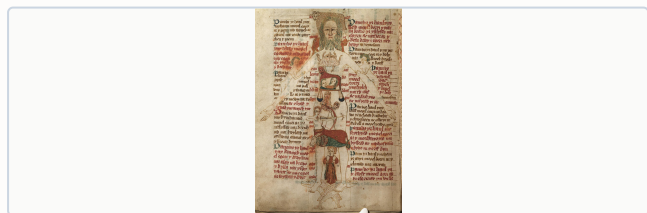
STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Advocated Observation:** Argued doctors should test ideas through experimentation rather than dogmatically copying ancient texts.
- **Imprisoned in 1277:** Punished by the Franciscan Order for holding "suspect novelties", deterring empirical science.
- **Symbol of Censorship:** Demonstrated the Church's unyielding enforcement of Galenic and theological orthodoxy.

SOURCE C ARCHIVAL RECORD & ANALYSIS

c.1250–c.1500

The 'Zodiac Man' (Homo Signorum, c.1380)



"The Zodiac Man (Homo Signorum) showing the 12 astrological signs that medieval physicians believed governed different bodily organs."

Provenance: MS Harley 3719, British Library, London, c.1380.

Hinge Question: Notice how each astrological constellation rules a specific part of the human body (such as Aries governing the head and Pisces governing the feet). Why did medieval physicians carry pocket almanacs and consult the position of the moon before performing bloodletting or surgery on a patient?

HISTORICAL CONCEPT

THEOLOGICAL DOGMA

Teleology & The Papal Monopoly on Knowledge

Why did the Christian Church adopt the medical treatises of Claudius Galen, a pagan doctor from 2nd-century imperial Rome? The crucial connection was teleology. Galen repeatedly argued in his treatises that every single bone, muscle, and organ in the human body had been purposefully crafted by a supreme, singular Creator. This design argument aligned so perfectly with Genesis that church leaders declared Galen's anatomical writings infallible sacred truth.

Key Causation: Questioning Galen became synonymous with heresy against God, freezing anatomical progress for over 1,300 years.

KEY ENQUIRY CHECK • SUPERNATURAL & RELIGIOUS EXPLANATIONS OF DISEASE (C1250–C1500)

CHECK YOUR UNDERSTANDING

1. Causal Recall: Which ancient civilization first introduced the idea of natural, humoral balance?

2. Historical Analysis: Name the ancient Roman physician who created the Theory of Opposites.

3. Evaluative Hinge: What four bodily fluids did the humoral theory claim dictated human health?

KT1.2: Rational Explanations: Hippocrates, Galen & The Four Humours (c1250–c1500)

Specification Anchor: Rational explanations: the Theory of the Four Humours and the Theory of Opposites; the miasma theory; the continuing influence of Hippocrates and Galen.

The Greek Foundations of Humoural Theory

[1.1] While religion explained why God permitted illness, medieval doctors needed a naturalistic, physical framework to explain how disease functioned inside the body. For this, European universities relied upon the ancient Greek physician Hippocrates (c. 460–c. 370 BC). Hippocrates made a revolutionary contribution by rejecting demonic possession and witchcraft, arguing instead that health and disease were entirely natural phenomena governed by bodily fluids.

[1.2] Hippocrates posited that the human body contained four primary liquids, termed the Four Humours: Blood (produced by the liver; associated with spring, air, and being hot and wet); Phlegm (produced by the brain and lungs; associated with winter, water, and being cold and wet); Yellow Bile or Choler (produced by the gall bladder; associated with summer, fire, and being hot and dry); and Black Bile or Melancholy (produced by the spleen; associated with autumn, earth, and being cold and dry). Good health required all four humours to remain in harmonious balance. If an individual developed an excess of one humour, or if a humour became putrefied, physical illness immediately resulted.

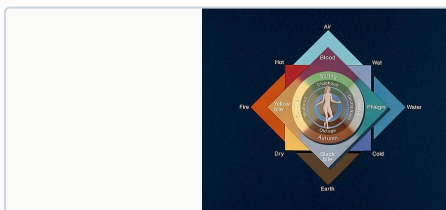
Galen & The Theory of Opposites

[2.1] In the 2nd century AD, the Roman imperial physician Claudius Galen expanded Hippocrates' humoural theory into a comprehensive clinical treatment methodology known as the Theory of Opposites (Contraria Contrariis Curantur). Galen argued that doctors should treat illnesses by counteracting the excessive qualities of the dominating humour with their direct opposites.

[2.2] Under this rational system, if a patient suffered from an excess of cold, wet phlegm (manifested as a shivering winter cold and sneezing), the physician prescribed warm, dry treatments such as eating hot chilli peppers, drinking ginger infusions, and resting by a roaring fire. Conversely, if a patient suffered from a high fever caused by an excess of hot, dry yellow bile or hot blood, the doctor prescribed cool cucumbers, cold baths, and bloodletting to lower internal heat. This system gave physicians a systematic logic for prescribing diets, herbal drinks, and purges.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

The Four Humours Wheel (Tacuinum Sanitatis, 14th c.)



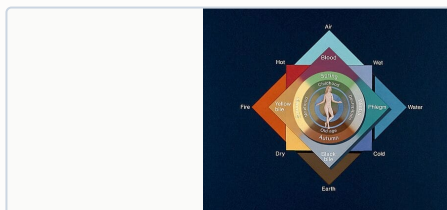
“The Four Humours Wheel: Blood (air/hot & wet), Phlegm (water/cold & wet), Yellow Bile (fire/hot & dry), and Black Bile (earth/cold & dry).”

Provenance: Tacuinum Sanitatis manuscript, 14th century, Österreichische Nationalbibliothek, Vienna.

Hinge Question: Study how each of the four humours (Blood, Phlegm, Yellow Bile, Black Bile) is linked to a season, an element, and specific qualities of heat and moisture. If a patient suffering from a cold and shivering was treated with hot spices and dry foods under Galen's Theory of Opposites, why did this belief in internal fluid balance prevent medieval physicians from searching for external causes of disease?

SOURCE B HISTORICAL PRIMARY EVIDENCE c1250–c1500

The Four Humours Wheel (Tacuinum Sanitatis, 14th c.)



“The Four Humours Wheel: Blood (air/hot & wet), Phlegm (water/cold & wet), Yellow Bile (fire/hot & dry), and Black Bile (earth/cold & dry).”

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KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS

the Four Humours: Blood (hot/wet), Phlegm (cold/wet), Yellow Bile (hot/dry), and Black Bile (cold/dry). Sickness was an internal imbalance of these fluids.

Galen's Theory of Opposites: A clinical method treating excess humours with contrary qualities (e.g. treating cold, wet phlegm with hot, dry peppers and spices).

Miasma Theory: The belief that poisonous, foul-smelling air from swamps, rotting waste, and corpses poisoned the body's humours upon inhalation.

CORE VOCABULARY

Miasma Theory & Pestilential Air

[3.1] Alongside internal humoral imbalance, the leading environmental explanation for epidemic disease was Miasma. Miasma was defined as corrupt, foul-smelling air filled with poisonous vapors. Medieval people believed that breathing in poisonous air directly corrupted the blood and unbalanced the humours inside the chest and heart. In medieval hospital and municipal records, this concept was described as "pestilential air," "corruption of the atmosphere," or "putrefaction."

[3.2] Miasma was believed to originate from stagnant marshes, open cesspits, unburied corpses, rotting animal carcasses, and dung heaps. Because bad smells were deeply associated with disease and sinfulness, medieval prevention heavily emphasized sweet odors. Wealthy citizens carried hollow pomanders filled with fragrant spices, held bunches of sweet herbs (posies) to their noses when walking through crowded streets, and burned aromatic incense in their homes.

Diagnostic Tools—Uroscopy & The Vademecum

[4.1] Because human dissection was forbidden, how did a university-trained medieval physician actually diagnose internal humoral imbalance? The cornerstone of clinical examination was Uroscopy (the inspection of urine). Physicians collected patient urine in a pear-shaped glass flask called a matula, designed to represent the shape of the human bladder. The doctor carefully examined the color, thickness, sediment, and smell of the sample, sometimes even tasting it, comparing it against a radial chart of 20 distinct urine shades.

[4.2] A deep red or golden urine indicated an excess of hot blood or choler; pale, watery white urine indicated an excess of cold phlegm. Doctors carried this chart inside their folding pocket Vademecum handbook alongside the Zodiac Man. While uroscopy involved empirical observation of physical bodily fluids, it completely locked doctors inside Galen's flawed humoral framework. Because doctors assumed every symptom was caused by humours rather than bacteria or physical organ failure, patient diagnosis remained fundamentally inaccurate.

SOURCE C ARCHIVAL RECORD & ANALYSIS

c1250–c1500

Physician Inspecting Urine Flask (Fasciculus Medicinae, 1491)



"The Medieval Urine Chart: a radial wheel showing twenty flasks of differing colours, which physicians examined, smelled, and sometimes tasted to diagnose humoral imbalance."

Provenance: Johannes de Ketham, Fasciculus Medicinae, Venice, 1491.

Hinge Question: Observe the physician holding up a glass matula (urine flask) to inspect the color, sediment, and clarity of the liquid against a radial wheel. How did uroscopy allow medieval doctors to diagnose internal humoral imbalance without performing surgery or human dissection?

KEY FIGURE

c.129–c.216 AD



Claudius Galen

ANCIENT ROMAN IMPERIAL PHYSICIAN

Roman doctor whose Theory of Opposites and teleological design argument formed the unquestioned foundation of medieval European medicine.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Theory of Opposites:** Developed treatments balancing the Four Humours using opposing qualities (cold phlegm treated with hot pepper).
- **Teleological Doctrine:** Argued every organ was purposefully designed by a single Creator, perfectly matching Genesis creation theology.
- **Anatomical Dominance:** Dissected apes and pigs, creating errors (e.g. 2-piece jawbone) that remained dogma for over 1,300 years.

HISTORICAL CONCEPT

RATIONAL PATHOLOGY

The Humoural Axis: Balance, Phlebotomy & Opposites

Medieval physicians viewed the human body as an organic microcosm governed by the Four Humours: Blood (spring/air/hot & wet), Yellow Bile (summer/fire/hot & dry), Black Bile (autumn/earth/cold & dry), and Phlegm (winter/water/cold & wet). Disease occurred when one humour became excessive or corrupt. Treatment required active subtraction via phlebotomy (veinsection, cupping, or leeches) or clinical purging using emetics and laxatives.

Exam Distinction: Humoural medicine was rational because it sought natural biological causes rather than demonology, yet remained medically ineffective.

KEY ENQUIRY CHECK • RATIONAL EXPLANATIONS: HIPPOCRATES, GALEN & THE FOUR HUMOURS (C1250–C1500)

CHECK YOUR UNDERSTANDING

1. Causal Recall: Why did the medieval Catholic Church actively promote the medical texts of Galen?

2. Historical Analysis: What punishment did the English friar Roger Bacon receive in 1277 for advocating experiments?

3. Evaluative Hinge: Where were leprosy sufferers banished to in medieval society?

KT1.3: Approaches to Prevention & Treatment: Rituals, Bleeding & Purging (c1250–c1500)

Specification Anchor: Approaches to prevention and treatment: religious and supernatural methods (prayer, fasting, pilgrimages); rational treatments (bloodletting, purging, herbal remedies, regimen sanitatis).

Spiritual Healing & Religious Penitence

[1.1] Because illness was widely regarded as God's punishment for sin, the first and most urgent response to sickness was spiritual. Sufferers were instructed to confess their sins to a parish priest and pray for divine forgiveness. The Church taught that without spiritual cleansing, physical treatments were entirely useless. Wealthy families paid for chanting monks to sing special intercessory Masses, while ordinary people fasted, gave alms to the poor, and lit votive candles before images of healing saints.

[1.2] Pilgrimages were among the most popular therapeutic actions in medieval England. Sick pilgrims walked hundreds of miles to shrines containing holy relics—such as the tomb of Saint Thomas Becket at Canterbury Cathedral, or the shrine of Our Lady at Walsingham. Sufferers touched reliquaries containing saints' bones, drank holy water, or purchased lead pilgrim badges as talismans. Miraculous cures reported at shrines reinforced the belief that divine intervention was far more powerful than physical medicine.

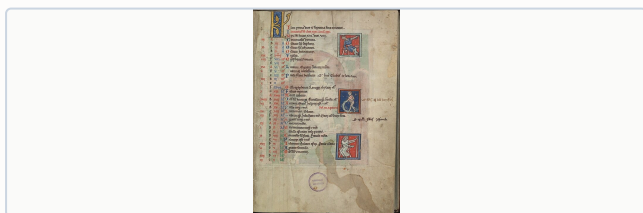
Phlebotomy—The Universal Humoural Treatment

[2.1] When physical treatments were administered, the most common and revered procedure was Phlebotomy (bloodletting). Based upon Galen's belief that blood was the dominant, warmest humour and the primary cause of fevers, physicians ordered regular bloodletting to lower internal bodily heat and restore equilibrium. Phlebotomy was so routine that monasteries set aside specific "bleeding weeks" several times a year where all monks were bled for preventative maintenance.

[2.2] Phlebotomy was performed using three distinct surgical techniques. The most common was vein-opening using a sharp, double-edged lancet called a fleam, collecting up to a pint of blood in a marked pewter bowl. Alternatively, for localized inflammation, practitioners used Cupping: heating a glass cup over a flame and placing it over scratched skin, creating a vacuum that drew blood to the surface. Finally, Leeching was utilized for delicate areas like the face or piles; medicinal leeches were placed on the skin to engorge themselves on blood. While intended to heal, excessive bloodletting drastically lowered blood pressure, induced fainting, and frequently caused fatal hypovolemic shock in weakened patients.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

Phlebotomy / Bloodletting Manuscript (MS Sloane 1977, c.1300)



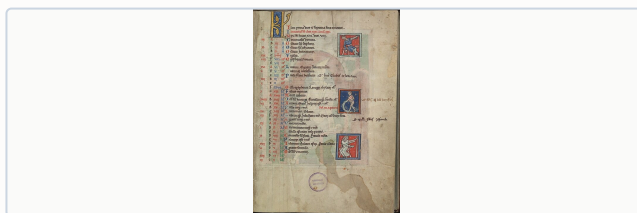
"A medieval physician opening a vein in the arm using a fleam while blood drains into a bowl, supervised by an attendant."

Provenance: British Library MS Sloane 1977, England, c.1300.

Hinge Question: Study the physician using a fleam (blade) to open a vein and collect blood in a bowl. Why did medieval people believe that releasing blood was an essential way to restore balance to the body and prevent fevers, even though it frequently weakened the patient?

SOURCE B HISTORICAL PRIMARY EVIDENCE c1250–c1500

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KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS

Phlebotomy: The practice of bloodletting using fleams, cupping glasses, or medicinal leeches to lower internal bodily heat and restore humoral balance.

the three methods of bloodletting: 1. Vein-opening with a fleam; 2. Cupping (heated glass vacuum); 3. Leeching (medicinal leeches drawing blood).

Purging: Using emetics (inducing vomiting) and strong laxatives (scammony, hellebore) or clysters (enemas) to clear corrupt humours from the digestive tract.

CORE VOCABULARY

Purging, Emetics & Herbal Theriac

[3.1] If bloodletting failed to rebalance the humours, physicians turned to Purging to evacuate corrupted digestive fluids. The digestive system was viewed as an internal furnace; if food putrefied, it generated toxic yellow and black bile. Doctors administered powerful emetics containing antimony or dried crushed beetles to induce violent vomiting. Alternatively, strong herbal laxatives made from scammony, senna, and hellebore were given to cause explosive diarrhoea, or clysters (enemas) were pumped into the rectum using pigs' bladders and reeds.

[3.2] Herbal medicine was the primary pharmaceutical therapy. Apothecaries mixed complex syrups, electuaries, and ointments using local herbs (mint, sage, chamomile, garlic) and expensive imported spices (cinnamon, cloves, pepper). The most famous universal cure-all was Theriac (treacle), an ancient compound containing over 60 ingredients, including crushed viper flesh, opium, and honey. Prescribed for poisons, fevers, and bites, Theriac was so trusted that physicians believed it could neutralize any humoral toxin.

Prevention & The Regimen Sanitatis

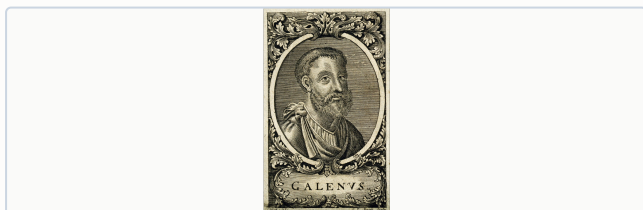
[4.1] Prevention was viewed as far superior to cure. In medieval thinking, maintaining personal health was a moral and physical duty. Wealthy nobles and high-ranking clerics commissioned custom copies of the Regimen Sanitatis (Rule of Health), a lifestyle guide originating from the medical school of Salerno. The guide advised daily moderation across six essential factors: air quality, exercise, diet, sleep, bowel evacuations, and emotional control.

[4.2] People were instructed to bathe regularly in warm herbal infusions, avoid damp night mists, sleep with windows shuttered against miasma, and eat foods tailored to their humoral complexion (e.g. moist fish for hot personalities). While these hygiene measures promoted general wellbeing, they provided zero protection against deadly bacterial infections like the plague. Because medieval treatments were based on balancing imaginary humours through aggressive bleeding and purging, medical care often hastened the death of the sick.

SOURCE C ARCHIVAL RECORD & ANALYSIS

c1250–c1500

Portrait of Claudius Galen (Engraving)



"Claudius Galen (AD 129–c. 216), whose clinical treatises on the Theory of Opposites, phlebotomy, and purging dominated European medicine for 1,400 years."

Provenance: Historical engraving after ancient Roman busts of Galen.

Hinge Question: Galen taught that the humours must be kept in constant equilibrium through balancing treatments. Why did his authoritative treatises lead doctors to prescribe violent purging (emetics and laxatives) alongside herbal compounds like Theriac, rather than searching for specific remedies for individual illnesses?

KEY FIGURE

c.460–c.370 BC



Hippocrates of Kos

ANCIENT GREEK PHYSICIAN & PHILOSOPHER

The "Father of Medicine" who separated disease from supernatural causes and pioneered clinical bedside observation.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Theory of the Four Humours:** Identified Blood, Phlegm, Yellow Bile, and Black Bile as the determinants of physical health.
- **Clinical Observation:** Instructed physicians to examine symptoms, pulse, and urine systematically rather than relying on omens.
- **The Hippocratic Oath:** Established an ethical code of conduct mandating that doctors protect patient confidentiality and "do no harm".

HISTORICAL CONCEPT

PREVENTIVE REGIMES

Regimen Sanitatis: Preventive Hygiene & The Six Non-Naturals

Medieval prevention centered on the Regimen Sanitatis, a set of dietary and behavioural rules originating in ancient Greek medicine. Doctors advised patients on the "Six Non-Naturals": air quality, food and drink, motion and rest, sleep and waking, excretion and retention, and passions of the soul. Wealthy citizens burned aromatic sweet woods (frankincense, lavender) to counteract miasma and bathed regularly in stews.

Key Evidence: Medieval people actively cared about hygiene; sickness was prevented by keeping the internal humours and external environment pure.

KEY ENQUIRY CHECK • APPROACHES TO PREVENTION & TREATMENT: RITUALS, BLEEDING & PURGING (C1250–C1500)

CHECK YOUR UNDERSTANDING

1. Causal Recall: What Greek physician originally created the Theory of the Four Humours?

2. Historical Analysis: Under Galen's Theory of Opposites, how would a feverish patient be treated?

3. Evaluative Hinge: What did physicians examine in a glass matula to deduce humoral balance?

KT1.4: Medical Care Providers & Monastic Hospitals: 'Care Not Cure' (c1250–c1500)

Specification Anchor: The roles of the physician, apothecary and barber surgeon; care in the home; the role of medieval hospitals.

The Elite University Physician

[1.1] In medieval England, access to healthcare was strictly dictated by wealth and social status. At the pinnacle of the medical hierarchy was the university-trained Physician (Medicus). Physicians studied for 7 to 10 years at ecclesiastical universities such as Oxford, Cambridge, Paris, or Bologna. Their training was entirely theoretical and literary; students memorised classical Latin translations of Galen, Hippocrates, and Islamic scholars like Avicenna and Rhazes. Physical contact with living sick people was almost non-existent during their degrees.

[1.2] Because training was lengthy and expensive, university physicians were exceedingly rare—it is estimated there were fewer than 100 fully qualified physicians in England before 1350. They were retained almost exclusively by royalty, wealthy aristocrats, and bishops. When treating a patient, the physician did not dress wounds or dispense medicine; he examined the urine flask (matula), took the pulse, consulted astrological charts, and wrote a Latin prescription dictating which humours required bleeding or purging.

Barber-Surgeons, Apothecaries & Domestic Healers

[2.1] For the vast majority of ordinary citizens, healthcare was delivered by craftsmen rather than university scholars. Practical, manual procedures were performed by Barber-Surgeons. Barber-surgeons were guild-trained through manual apprenticeships. Armed with razors, fleams, probes, and bone-saws, they pulled teeth, stitched lacerations, set broken limbs, lanced boils, and performed bloodletting. Because they worked with their hands and shed blood, university physicians regarded them with social contempt, viewing them as uneducated tradesmen.

[2.2] Meanwhile, medicines were formulated by Apothecaries. Organized in merchant guilds, apothecaries mixed herbal compounds, ointments, and syrups, stocking both domestic English herbs and imported spices. Finally, the true frontline of medieval healthcare was Care in the Home, provided by women, mothers, and local "wise women." Women brewed herbal teas, dressed minor wounds, and served as village midwives. Wise women possessed generations of inherited folklore regarding local medicinal plants, offering cheap, accessible relief to peasant communities.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

Medieval Surgeon Suturing a Head Wound (14th c.)



"A guild-trained surgeon physically stitching a patient's severe head wound with needle and thread, recorded in a 14th-century Latin surgical treatise."

Provenance: Wellcome Collection MS L0037333, 14th century surgical treatise.

Hinge Question: Observe the surgeon physically stitching a wound without any antiseptics or anaesthetics. Why were barber-surgeons considered socially inferior to university-trained physicians in the Middle Ages, despite being the only practitioners performing practical, hands-on treatment?

SOURCE B HISTORICAL PRIMARY EVIDENCE c1250–c1500

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"A guild-trained surgeon physically stitching a patient's severe head wound with needle and thread, recorded in a 14th-century Latin surgical treatise."

Provenance: Wellcome Collection MS L0037333, 14th century surgical treatise.

Hinge Question: Observe the surgeon physically stitching a wound without any antiseptics or anaesthetics. Why were barber-surgeons considered socially inferior to university-trained physicians in the Middle Ages, despite being the only practitioners performing practical, hands-on treatment?

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS

the training of a medieval university physician: 7 to 10 years of theoretical book-learning at universities like Oxford or Paris, studying Latin translations of Galen and Hippocrates, with almost zero hands-on clinical practice.

the role of a barber-surgeon: Guild-apprenticed craftsmen who performed manual surgery: bloodletting, pulling teeth, suturing wounds, lancing boils, and amputations without anaesthetics.

What did apothecaries do: Guild merchants who mixed and sold herbal remedies, ointments, and syrups based on traditional recipes and Galen's Theory of Opposites.

CORE VOCABULARY

The Medieval Monastic Hospital

[3.1] Between 1100 and 1500, over 1,200 hospitals were established across England. However, a medieval hospital was fundamentally different from a modern medical center. The English word "hospital" derived from the Latin hospitalitas (hospitality). Hospitals were religious charitable foundations funded by royal endowments or wealthy benefactors seeking to reduce their time in Purgatory. They were operated entirely by Catholic religious orders—monks, canons, and Augustinian nuns—not by doctors.

[3.2] Famous foundations like St Bartholomew's (founded in London in 1123 by Rahere) and St Thomas's housed travelers, the elderly, and the impoverished sick. The central ethos was "Care Not Cure." Patients were washed, provided with clean bedding, warmed by large open hearths, fed nourishing broths and ale, and comforted through continuous prayer. However, no medical treatment, surgery, or pharmacological cure was attempted. Wards were laid out like church chapels, with beds arranged in rows pointing toward a high altar so that bedridden patients could witness the elevation of the Host during Mass and pray for their salvation.

Strict Exclusion & The Lazar House

[4.1] Because the primary goal of monastic hospitals was peace, prayer, and hospitality, they strictly enforced entrance criteria. Hospital regulations explicitly barred patients suffering from contagious infectious illnesses, leprosy, or mental insanity, as well as pregnant women. Authorities feared that infectious sufferers would disrupt monastic prayers and contaminate the spiritual purity of the house.

[4.2] Instead, infectious diseases were segregated outside urban centers. Sufferers of leprosy were consigned to Lazar Houses (leprosaria), such as St Giles in London or Sherburn Hospital in Durham. In summary, medieval institutional care offered immense Christian charity, shelter, and spiritual solace to the poor, but contributed nothing to anatomical research or clinical cures. Effective medical treatment remained absent from the medieval hospital.

KEY FIGURE

1307–1392



John of Arderne

MASTER SURGEON OF THE HUNDRED YEARS WAR

Renowned English military surgeon who developed innovative surgical techniques for treating anal fistulae in knights.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Fistula-in-ano Procedure:** Devised a revolutionary surgical incision that achieved a 50% survival rate without fatal hemorrhage.
- **Empirical Practice:** Founded the Guild of Surgeons (1376) and advocated for practical battlefield experience over university theory.
- **Bedside Demeanour:** Wrote *Practica*, advising surgeons to dress soberly, soothe patient anxieties, and agree fees in advance.

SOURCE C ARCHIVAL RECORD & ANALYSIS

c1250–c1500

Hôtel-Dieu Hospital Ward (Livre de Vie Active, c.1482)



"The main ward of the Hôtel-Dieu in Paris: Augustinian nuns tending to rows of patients sharing beds, providing spiritual comfort and rest."

Provenance: Livre de Vie Active des Religieuses de l'Hôtel-Dieu de Paris, Jean Henry, c.1482.

Hinge Question: Look at the nuns tending to patients in the ward, with an altar visible in the background. Why was the primary purpose of a medieval hospital hospitality and salvation rather than medical cure, and why were infectious patients strictly excluded?

HISTORICAL CONCEPT

INSTITUTIONAL CARE

'Care Not Cure': The Monastic Hospital Philosophy

Medieval English hospitals were not medical centers designed for surgery or infectious healing. Run by monastic religious orders, their primary purpose was spiritual hospitality and palliative care. Priests and nuns provided clean bedding, warmth, simple food, and daily prayer so patients could die in a state of Christian grace. Infectious lepers, lunatics, pregnant women, and terminal sufferers were routinely turned away.

Core Specification Anchor: "Care Not Cure" defines the institutional purpose of all 500+ medieval English monastic hospitals.

KEY ENQUIRY CHECK • MEDICAL CARE PROVIDERS & MONASTIC HOSPITALS: 'CARE NOT CURE' (C1250–C1500)

CHECK YOUR UNDERSTANDING

1. Causal Recall: What surgical treatment involved opening a vein with a fleam or using leeches?

2. Historical Analysis: What famous herbal cure-all contained over 60 ingredients including crushed viper flesh?

3. Evaluative Hinge: Under Galen's Theory of Opposites, why were emetics administered to feverish patients?

KT1.5: Case Study: Dealing with the Black Death (1348–1349)

Specification Anchor: Case study: dealing with the Black Death, 1348–49; approaches to treatment and attempts to prevent its spread.

The Arrival & Deadly Symptoms of the Pestilence

[1.1] In June 1348, ships from Gascony carrying wine docked at the port of Melcombe Regis in Dorset. Unbeknownst to the sailors, their cargo harboured black rats carrying fleas infected with the deadly bacterium *Yersinia pestis*. Within weeks, the catastrophic epidemic known as the Black Death broke out, sweeping relentlessly across England along trade routes and engulfing London by autumn. Between 1348 and 1350, the pestilence decimated the country, killing an estimated 30% to 50% of the entire population—over 1.5 million people.

[1.2] The disease manifested in two terrifying clinical forms. The most prevalent was Bubonic Plague, transmitted by the bites of infected black rat fleas. Victims suffered from agonizing high fevers, shivering, violent vomiting, dark subcutaneous hemorrhages (causing purple blotches called "God's tokens"), and large, painful inflammatory swellings of the lymph nodes in the groin, armpits, and neck, termed buboes. Buboes swelled to the size of apples, blackened, and oozed foul pus; untreated, victims died in agony within 3 to 5 days. The second, even deadlier form was Pneumonic Plague, which occurred when the bacterium infected the lungs. Spread directly through the air via coughs and breath, pneumonic plague caused victims to cough up black blood, killing almost 100% of sufferers within 48 hours.

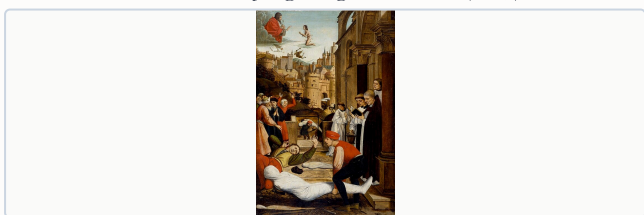
Explaining the Apocalypse—God, Astrology & Miasma

[2.1] Entirely ignorant of microscopic bacteria or flea vectors, medieval society explained the catastrophe through established religious and humoral doctrines. The primary explanation was Supernatural: the Church preached that God had sent the plague as divine retribution to punish humanity for its wickedness, pride, and greed. Many believed the end of the world had arrived. Simultaneously, astrologers pointed to the March 1345 planetary conjunction of Saturn, Jupiter, and Mars in Aquarius, claiming celestial alignment had corrupted the atmosphere.

[2.2] On an earthly level, Miasma theory was universally blamed. Physicians claimed that foul, pestilential air caused by unburied decomposing bodies, stagnant cesspits, animal dung, and poisonous vapors rising from earthquakes had entered victims' bodies, completely corrupting their humours. In mainland Europe, panic also sparked xenophobic scapegoating; Jews were falsely accused of poisoning drinking wells, leading to horrific pogroms, though in England, the Jewish population had already been expelled by King Edward I in 1290.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

Citizens of Tournai Burying Plague Victims (1349)



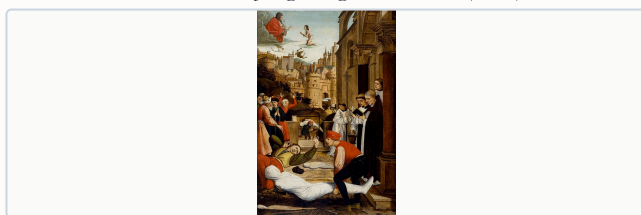
"The citizens of Tournai carrying wooden coffins to mass communal plague burial pits during the devastating Black Death epidemic of 1349."

Provenance: Illumination by Pierart dou Tiel, Gilles Li Muisis chronicle, MS 13076-77, Bibliothèque Royale de Belgique, Brussels, 1349.

Hinge Question: Study the sheer number of wooden coffins being carried simultaneously to a mass burial trench outside the city walls. What does this image show about how the rapid spread and terrifying death toll of the Black Death overwhelmed traditional parish burial customs and churchyards?

SOURCE B HISTORICAL PRIMARY EVIDENCE c1250–c1500

Citizens of Tournai Burying Plague Victims (1349)



"The citizens of Tournai carrying wooden coffins to mass communal plague burial pits during the devastating Black Death epidemic of 1349."

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Hinge Question: Study the sheer number of wooden coffins being carried simultaneously to a mass burial trench outside the city walls. What does this image show about how the rapid spread and terrifying death toll of the Black Death overwhelmed traditional parish burial customs and churchyards?

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS

the Black Death enter England: June 1348 at the port of Melcombe Regis in Dorset, arriving on trade ships from Gascony carrying plague-infected black rats and fleas.

the two strains of the Black Death: 1. Bubonic plague (transmitted by rat flea bites, causing agonizing groin/armpit buboes); 2. Pneumonic plague (airborne via coughs/breath, attacking lungs, nearly 100% fatal).

believed causes of the Black Death in 1348: God's punishment for sin, the 1345 planetary conjunction of Mars, Jupiter, and Saturn in Aquarius, and corrupt miasma from rotting filth and unburied corpses.

CORE VOCABULARY

Desperate Treatments & Religious Panic

[3.1] Because people believed God was punishing them, religious responses were frenzied. The King and bishops ordered daily church services, fasting, and massive public religious processions. Most extreme were the Flagellants: sects of penitent men who marched from town to town stripped to the waist, whipping themselves with iron-tipped scourges to appease God's anger. Tragically, these large public gatherings and bleeding flagellants actively accelerated the transmission of pneumonic plague and flea vectors between communities.

[3.2] Medical treatments were completely ineffective and frequently agonizing. Physicians and barber-surgeons applied heated cupping glasses or lanced swollen buboes with red-hot irons to drain "corrupt humours," which caused excruciating pain and deadly secondary blood infections. Other doctors strapped live, plucked chickens or toads to the buboes, believing the creatures would draw out the poison. Patients were force-fed Theriac or made to drink crushed emeralds and vinegar, while homes were sealed to prevent bad air from entering.

Civic Action, Edward III & Social Collapse

[4.1] To combat miasma, local authorities and ordinary people made desperate attempts at environmental prevention. Citizens lit large bonfires in public squares, burned fragrant rosemary and incense, and continuously carried sweet posies or held sponges soaked in vinegar to their faces. Some towns attempted quarantine: Gloucester shut its gates to outsiders, though travelers simply carried the disease along country lanes. In London, parish churchyards overflowed within weeks, forcing the Bishop of London to consecrate emergency communal trench pits at East Smithfield, where bodies were stacked five deep.

[4.2] In April 1349, King Edward III intervened, writing a stern mandate to the Mayor of London complaining that the streets were choked with human faeces, rotting animal dung, and putrefying entrails, generating a stench that was poisoning citizens. He ordered the streets to be thoroughly cleaned to clear the miasma. However, because authorities had no police force or civil service, and because the true vector—fleas on black rats—remained entirely unknown, street cleaning could not halt the pandemic. The Black Death wiped out up to half of England's population, shattering feudal social structures and proving the total helplessness of medieval medicine.

SOURCE C ARCHIVAL RECORD & ANALYSIS

c.1250–c.1500

Plague Victim with Groin and Armpit Buboes (1497)



"A bedridden plague victim showing painful inflamed lymph swellings (buboes) in the groin and neck, surrounded by weeping attendants."

Provenance: Woodcut of St. Sebastian interceding for plague victims, Augsburg, 1497.

Hinge Question: Observe the painful, dark swellings (buboes) in the groin, armpits, and neck of the plague victim. Why did medieval physicians believe these buboes were proof that poisonous humours were corrupting the body, and why did desperate treatments like lancing them or applying toads fail to cure the disease?

KEY FIGURE

c.1300–1368



Guy de Chauliac

PAPAL PHYSICIAN TO POPE CLEMENT VI

Leading French surgeon who treated victims of the Black Death at Avignon and authored the surgical textbook *Chirurgia Magna*.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Identified Plague Types:** Distinguished clinically between pneumonic plague (blood-spitting) and bubonic plague (buboes).
- **Protected the Papacy:** Advised Pope Clement VI to isolate himself between two roaring fires in Avignon, successfully saving his life.
- **Admitted Medical Powerlessness:** Openly recorded in 1348 that doctors were helpless, as "all medicines and bloodletting availed nothing".

HISTORICAL CONCEPT

EPIDEMIOLOGY

Astrology, Miasma & Contagion: The 1348 Conjunction

When the Black Death arrived in June 1348, medieval society deployed three concurrent causal models. Astrologers blamed the March 1345 planetary conjunction of Saturn, Jupiter, and Mars in Aquarius. Churchmen declared it God's vengeance for England's pride and sinful dress. Common folk suspected poisoned wells and foul miasma. Local authorities reacted with quarantine (40-day isolation in Gloucester) and cemetery bans outside city limits.

Historiographical Reality: Medieval authorities attempted practical public health measures, but lacked biological knowledge of flea vectors.

KEY ENQUIRY CHECK • CASE STUDY: DEALING WITH THE BLACK DEATH (1348–1349)

CHECK YOUR UNDERSTANDING

1. Causal Recall: What was the primary purpose of a medieval monastic hospital?

2. Historical Analysis: Which of the following was strictly excluded from medieval monastic hospitals?

3. Evaluative Hinge: What medical practitioners performed practical surgery and bloodletting in medieval England?

MEDIEVAL MEDICINE IN BRITAIN: CHRONOLOGY & DISCIPLINARY MASTERY

(C1250–C1500)

Comprehensive revision index of pivotal chronology, core specification concepts, and Edexcel examination question frameworks.

MEDIEVAL MEDICINE IN BRITAIN • PIVOTAL CHRONOLOGICAL SEQUENCE & CAUSAL TURNING POINTS			PAPER 1 CHRONOLOGY RECALL
Date	Pivotal Milestone	Historical Significance & Causal Consequence	
1123	Rahere founds St Bartholomew's Hospital in London ('Care Not Cure').	Rahere founds St Bartholomew's Hospital in London ('Care Not Cure').	
c.1242	Ibn al-Nafis discovers the pulmonary transit of blood in Damascus.	Ibn al-Nafis discovers the pulmonary transit of blood in Damascus.	
1277	Roger Bacon imprisoned by Church authorities for advocating empirical observation.	Roger Bacon imprisoned by Church authorities for advocating empirical observation.	
1345	Planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for plague.	Planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for plague.	
1348	The Black Death strikes Melcombe Regis, killing 30–45% of England's population.	The Black Death strikes Melcombe Regis, killing 30–45% of England's population.	
1376	Guild of Surgeons established in London, separating surgeons from barbers.	Guild of Surgeons established in London, separating surgeons from barbers.	
c.1380	MS Harley 3719 illustrates the 'Zodiac Man' (Homo Signorum) for bloodletting.	MS Harley 3719 illustrates the 'Zodiac Man' (Homo Signorum) for bloodletting.	
1400	Over 500 monastic hospitals operate across England under religious orders.	Over 500 monastic hospitals operate across England under religious orders.	

CORE SPECIFICATION CONCEPTS (C1250–C1500)

- **Teleology & The Papal Monopoly on Knowledge:** Questioning Galen became synonymous with heresy against God, freezing anatomical progress for over 1,300 years.
- **The Humoural Axis:** Humoural medicine was rational because it sought natural biological causes rather than demonology, yet remained medically ineffective.
- **Regimen Sanitatis:** Medieval people actively cared about hygiene; sickness was prevented by keeping the internal humours and external environment pure.
- **'Care Not Cure':** "Care Not Cure" defines the institutional purpose of all 500+ medieval English monastic hospitals.
- **Astrology, Miasma & Contagion:** Medieval authorities attempted practical public health measures, but lacked biological knowledge of flea vectors.

EDEXCEL PAPER 1 EXAMINATION FRAMEWORK

SECTION B • 32 MARKS

Q3: Explain one similarity / difference between... [4 Marks]
Strategy: Identify 1 valid conceptual link across eras • Support with precise detail from both specified periods.

Q4: Explain why [change/continuity occurred]... [12 Marks]
Strategy: 3 PEEL paragraphs • Use 2 stimulus points + own knowledge • Link with causal connectives.

Q5/Q6: Evaluative Essay [16 Marks + 4 SPaG]
Strategy: Criteria-led introduction • 3 balanced PEEL paragraphs weighing stated factor vs alternatives • Weighed verdict.

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